

Dear Deputy Secretary and ASTP/ONC Leadership,

Thank you for the opportunity to provide input on the integration of artificial intelligence into clinical care. As a physical therapist, educator, and academic leader responsible for curriculum development and professional formation, I support the continued acceleration of AI adoption in healthcare. However, sustained success will depend on thoughtful governance, high-quality data infrastructure, and deliberate investment in workforce preparation.

I agree that data fragmentation remains a central structural barrier. AI tools are only as strong as the datasets on which they are trained. If health data remain siloed across systems, specialties, and populations, we risk perpetuating bias and widening disparities rather than reducing them. Responsible interoperability standards, data quality requirements, and safeguards for privacy must remain foundational. Rapid adoption without rigorous data standards may yield short-term innovation but long-term erosion of trust. This loss of trust is of great concern to me.

Clear accountability structures are equally essential. Where clinicians retain decision-making authority, responsibility should align with professional judgment, consistent with existing standards of practice. However, as AI tools move toward more autonomous workflows, accountability must be shared between deploying organizations and product developers. Aligning accountability with decision-making authority will protect both patients and practitioners while maintaining clarity in governance. The human mind must be included in the decision making algorithm.

From an educational perspective, I would urge HHS to recognize that AI integration in clinical care necessitates parallel investment in healthcare education. AI literacy should not be treated as an optional technical competency; it is rapidly becoming a core professional skill. Healthcare students must be trained to critically evaluate AI outputs, understand algorithmic limitations, recognize automation bias, and apply tools within scope-of-practice boundaries. Without this foundation, even well-designed tools may be misapplied.

Federal support for AI workforce readiness could take several forms:

- Competitive grants to healthcare education programs to develop AI-integrated curricula
- Interprofessional pilot programs that study AI implementation in clinical training environments
- Scholarships or loan forgiveness programs for students pursuing advanced training in health informatics, clinical data science, or AI governance
- Faculty development funding to ensure educators themselves are prepared to teach AI competency responsibly

Such investments would help ensure that AI adoption enhances rather than outpaces clinical education and professional standards.

Finally, reducing clinician administrative burden through validated AI tools (such as ambient documentation systems) represents a high-value opportunity. Burnout is not merely a workforce concern; it is directly tied to patient safety and quality of care. Supporting the adoption of evidence-based administrative AI tools, particularly in resource-constrained and rural settings, could meaningfully improve care delivery capacity while preserving clinician engagement.